

Abandoning restrictions should not mean abandoning the vulnerable. We need policies to help those who remain at risk.

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The government has told us that we must “learn to live with the virus”. It is undoubtedly true that coronavirus is not going to disappear any time soon. However, a sizeable minority of people cannot learn to live with a virus that continues to pose a deadly risk.

I suffer from panhypogammaglobulinemia. This unpronounceable condition was triggered by the chemotherapy drugs that I received a decade ago. The condition means that I produce an extremely weak antibody response to viruses and vaccines.

Hence, despite having had a double dose of the vaccine, every medical professional with whom I have spoken recommends that I continue to avoid contracting the virus. This avoidance has been relatively easy for me to date. The most significant hardships have been isolation and the mental strain of worrying about the ongoing existential threat posed by the virus. However, kind friends and neighbours continue to help us to remain safe and I am very fortunate to be able to do so. Others are not so lucky.

And everything gets worse now the English government plans to lift all restrictions. At this point, it will become people's "personal choice" as to whether they wish to avoid the virus. The expectation that thousands of extremely vulnerable people will head back into poorly ventilated workplaces, using over-crowded public transport, with no masking, is a horror show. As this behaviour is re-normalised, those still at risk will be branded paranoid and offered "wellbeing" sessions to alleviate anxiety.

As the old saying goes, though: it's not paranoia if they are really after you. To date, 60% of deaths from the virus in England have been of disabled people. We need to protect this group if we are to open up again.

Indeed, we generally overlook two things about the rhetoric of "learning to live with the virus". First: what if, instead, "learning to live with the virus" meant that we adapted our ideas of liberty to protect the people who would otherwise die and make *that* the priority? At present, this is not what is meant. At the moment, "learning to live with the virus" means tolerating the deaths of the vulnerable as collateral.

Second: what if "learning to live with the virus" meant "providing help and assistance to the vulnerable, so that they can remain safe"? I have not yet seen any government plan to offer such help. But support for home working and furlough-esque schemes for the clinically vulnerable could offer some protection.

Of course, such schemes would also offer the unappealing prospect of indefinite isolation for the vulnerable. Isolation of this kind leads to stress, worry, and mental health problems. However, the actual risk of physical harm from the virus, coupled with the mental health damage from being placed in harm's way, must also not be overlooked. We are also too quick to conflate "indefinite" with "permanent". New treatments are on the way. Synthetic antibody cocktails can be administered to the vulnerable when they are through clinical trials. Indeed, for my condition, I already inject antibodies four days per week. The Covid equivalent is coming. We just need more time. Politicians could buy us such time if they put in place policies now.

In the rush to give everybody the freedom from masks, we should not remove disabled people's freedom to live. Learning to live with the virus should also mean supporting the vulnerable until the world is a safer place once more.